STANDARD OPERATING PROCEDURE			P-4600	
	SARPY COUNTY SHERIFF'S OFFICE PAPILLION * NEBRASKA **GREG LONDON** **SHERIFF**	DATE ISSUED 06/01/06	DATE REVISED **01/24/25**	DATE REVIEWED 02/02/25
		APPROVED: *GREG LONDON*		
CHAPTER TITLE ROAD PATROL	RESOURCES N.R.S. 71-906 THROUGH 71-921			
SUBJECT EMERGENCY PROTECTIVE CUSTODY	CROSS REFERENCE G-6300 AUTHORIZED RESTRAINTS P-4710 TRANSPORTATION OF SUSPECTS			

I. Purpose:

The purpose of this policy is to establish procedures for taking mentally ill and dangerous persons into emergency protective custody.

II. Policy:

The Sarpy County Sheriff's Office shall investigate and take appropriate action in situations involving persons suspected of being mentally ill and dangerous as defined by the Nebraska Mental Health Commitment Act.

III. Definitions:

A. MENTALLY ILL: Mentally ill means having a psychiatric disorder that involves a severe or substantial impairment of a person's thought processes, sensory input, mood balance, memory, or ability to reason which substantially interferes with such person's ability to meet the ordinary demands of living or interferes with the safety or well-being of others (N.R.S. 71-907).

B. MENTALLY ILL AND DANGEROUS PERSON: Mentally ill and dangerous person means a person who is mentally ill or substance dependent and because of such mental illness or substance dependence presents:

1. A substantial risk of serious harm to another person or persons within the near future as manifested by evidence of recent violent acts or threats of violence or by placing others in reasonable fear of such harm; or

2. A substantial risk of serious harm to himself or herself within the near future as manifested by evidence of recent attempts at, or threats of, suicide or serious bodily harm or evidence of inability to provide for his or her basic human needs, including food, clothing, shelter, essential medical care, or personal safety (N.R.S. 71-908).

IV. Procedures:

A. Mentally Ill and Dangerous Person-Deputy Responsibilities: Only a law enforcement officer possesses the legal authority to place a person in emergency protective custody (EPC). Emergency protective custody occurs in the jurisdiction where the subject is contacted by law enforcement even if a suicide attempt was made in another jurisdiction. If a deputy encounters a party meeting the criteria of mentally ill and dangerous, and the individual is 19 years of age or older, he or she shall adhere to the following procedures:

1. The deputy shall take immediate measures to safeguard the subject (including placing the party in authorized restraints-see G-6300 Authorized Restraints).

2. The deputy shall provide for any immediate medical attention required.

3. If a subject meets the criteria for emergency protective custody, the subject shall NOT be allowed to voluntarily commit himself or herself. An EPC shall be completed.

4. The deputy should attempt to obtain corroborating information and evidence from witnesses, relatives, or mental health professionals familiar with the subject.

5. The deputy shall contact UNMC-Psychiatric Emergency Services (PES) or Lasting Hope Recovery Center (LHRC) and provide details of the incident. The deputy should note what staff member they spoke with at PES or LHRC. If PES and LHRC do not have availability, the deputy shall transport the individual to the closest Emergency Department to be placed in Emergency Protective Custody provided the individual does not require medical clearance.

6. The deputy shall transport the individual to the closest Emergency Department if the individual needs to be medically cleared. Medical clearance is required for the following:

(1) Obvious overdose.
(2) Bleeding.
(3) Vomiting.
(4) Obvious physical injury.
(5) Severe head or chest pain.
(6) Substantial drug or alcohol impairment.

~~7.~~ The deputy shall transport the subject to the appropriate location (see P-4710 Transportation of Suspect). If the deputy transports the individual to LHRC he or she shall use the Law Enforcement Personnel entrance. If transporting to

UNMC-PES, the individual shall be taken in the Emergency Department of UNMC entrance.

8. The deputy shall complete the following:

a) An Emergency Protective Certificate

(1) If possible, the subject shall receive a copy of "The Rights of a Person Involved in a Commitment Proceeding."

(2) The deputy shall photocopy the EPC form (specifically the first page unless the Physician Observation form is completed) and leave the original form with the hospital.

(3) Copies of the EPC form shall be included with the incident report and submitted to transcription.

b) An EPC incident report: The deputy shall obtain an additional Report # for the EPC itself and shall cross reference it with the Report # for the original call. The EPC incident report shall **not** be referred to in any other report. The confidentiality of the EPC shall be maintained and no information regarding the EPC shall be released to anyone other than a licensed mental health professional or authorized person referred to in NRS 71-961.

c) Parties who are 18 years of age, and not current wards of the state, are to be EPC'ed following the adult EPC procedures, however an attempt shall first be made to place the party at Immanuel Hospital. If no beds are available at Immanuel or if the party requires medical clearance (see above IV. A. 6), the party shall be transported to the nearest hospital.

d) Complete a Mental Health Worksheet and include with the report.

B. Mentally Ill and Dangerous Juvenile (17 years of age and under) -Deputy Responsibilities: For purposes of this policy, a "qualifying juvenile" means a youth for whom a deputy has probable cause to believe is mentally ill and dangerous, as defined by NRS 71-908, who is either (1) 17 years of age or younger, or (2) is 18 years of age and a ward of the state.

1. Voluntary committals are preferable over placing a juvenile into emergency protective custody under NRS 43-248(3) (also referred to as "temporary custody" in the Nebraska Juvenile Code). "Juvenile EPCs" remove parental rights and make the juvenile a temporary ward of DHHS. Placing a juvenile into temporary custody shall be a last resort in the deputy's discretion.

2. In the case of a voluntary committal for a qualifying juvenile, deputies may transport the juvenile to a hospital and have the parent/guardian meet the deputy at the hospital to complete the voluntary committal.

3. If a deputy determines that a qualifying juvenile should be taken into temporary custody according to NRS 43-248(3), the deputy shall adhere to the following procedures:

a) The deputy shall take immediate measures to safeguard the juvenile (including placing the party in authorized restraints).

b) The deputy shall provide for any immediate medical attention required. Rescue personnel should be summoned and the juvenile should be transported to the nearest emergency room if he or she is suffering from any injury or other medical condition.

c) The Department of Health and Human Services shall be contacted on all juvenile EPC's and be provided the details of the incident. The deputy shall document the name of the DHHS Representative.

d) If no medical treatment is required and the juvenile is in the field, the deputy may call Immanuel Hospital (402-572- 2225) to determine bed space availability. If no beds are available, the deputy will transport the juvenile to the closest hospital (see P-4710 Transportation of Suspects).

e) If hospital personnel try to require the deputy to stay as the child's advocate, the deputy shall remind hospital personnel that DHHS is the advocate and custodian for the child and may provide them with DHHS's number (800- 652-1999).

f) The deputy shall also complete the following steps:

(1) The deputy shall fill out an Affidavit of Emergency Admittance Pursuant to Certificate of Peace Officer ("Affidavit").

(a) The deputy shall have the Affidavit notarized.
(b) The original Affidavit shall be left with hospital staff.

(2) The deputy shall photocopy the Affidavit and include a copy with the incident report.

(3) The deputy shall email a copy of the Affidavit to:

(a) The County Attorney's Office.
(b) The Department of Health and Human Services.

(c) The Juvenile Court Judges.

(4) The deputy shall create an incident report: The deputy shall obtain an additional "Report #" for the committal itself and shall cross-reference it with the "Report #" for the original call. The incident report shall **not** be referred to in any other report. The confidentiality of the committal shall be maintained and no information regarding the committal shall be released to anyone other than a licensed mental health professional or authorized person referred to in NRS 71-961.

4. Qualifying juveniles, who are in the custody of the Department of Health and Human Services, or the Sarpy County Juvenile Justice Center (JJC), Children at Risk Education Program (CARE) by court order, shall be taken into temporary custody according to the procedures described in Section B.3. above. The juvenile's parents shall NOT be allowed to voluntarily commit this juvenile unless permission is obtained from an on-scene supervisor or command officer.

5. If the juvenile is not committed, but the nature of the contact with the juvenile included the risk of harm to himself or herself, an information report shall be completed and the report shall be forwarded to the Juvenile Services Division for review.

6. Complete a Mental Health Worksheet and include with the report.

C. **Assessment, Support And Prevention (ASAP) Team:** Heartland Family Services provides an ASAP team to provide assistance to law enforcement in determining the need for mental health treatment for individuals, including juveniles and adults. The ASAP team can assist in resolving crisis situations or can assist "at risk" individuals who do not meet EPC criteria. A deputy may contact the ASAP team directly or request the ASAP team through Communications. The ASAP team made be utilized under the following conditions.

1. If an individual meets the criteria for an EPC, the ASAP team need not be requested, but the ASAP team may be contacted to provide additional screening and recommendations.

2. If a deputy is unsure if an individual meets the criteria for an EPC, the ASAP team may be requested to assist in making the determination. The ASAP team will provide a written recommendation regarding the individual's mental health status and needs.

3. If the ASAP team advises the individual needs immediate treatment, the individual SHALL be EPC'd.

4. If the ASAP teams advises the individual does not need immediate treatment, but the deputy believes an EPC is warranted, the deputy shall request his or her supervisor respond to the scene to determine the course of action.

5. If deputies encounter an individual who does not meet the criteria for an EPC but they feel the individual may not be safe enough to leave alone, the ASAP team may be contacted to provide referrals for services and to contact the individual's family members for support. The ASAP team will also provide follow-ups within twenty-four (24) hours to determine if the individual requires more services.

6. If an individual has not met the criteria for EPC, the deputy may transport the individual to the Heartland Family Services Crisis Center. The staff will intake the individual and provide services. This arrangement would be strictly voluntary by the individual and may be in addition to ASAP recommendations.

7. Complete a Mental Health Worksheet with details of the call in lieu of a report. Document ASAP recommendation.